

--SUMMARY--

Decision No. 1059/21

12-Aug-2021

S.Peckover

- Permanent impairment {NEL}
- Recurrences (compensable injury) (strains and sprains) (lumbar)

No Summary Available

11 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w3921n]

Style of Cause:

Neutral Citation: 2021 ONWSIAT 1260



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 1059/21

BEFORE:

S. Peckover: Vice-Chair

HEARING:

July 13, 2021 at Toronto
Written

DATE OF DECISION:

August 12, 2021

NEUTRAL CITATION:

2021 ONWSIAT 1260

DECISION UNDER APPEAL:

WSIB Appeals Resolution Officer (ARO) F. Amorim dated
November 2, 2017

APPEARANCES:

For the worker:

D. Chartrand, Union Representative

For the employer:

M. Hamilton, Paralegal

Interpreter:

Not Required

505 University Avenue 7th Floor
Toronto ON M5G 2P2

505, avenue University, 7^e étage
Toronto ON M5G 2P2

REASONS

(i) Introduction

- [1] The worker appeals a decision of the ARO F. Amorim dated November 2, 2017, which concluded that he was not entitled to recurrences of low back pain which occurred in August 2015 and December 2016.

(ii) Issues

- [2] The issues under appeal are as follows:

- [3] Entitlement for a recurrence of a low back injury which occurred in August 2015, and is claimed to be a recurrence of a prior compensable injury which occurred on February 6, 2006 or in April 2006; and

1. Entitlement for a recurrence of a prior compensable injury which occurred on December 22, 2016 and is claimed to be a recurrence of a prior compensable injury which occurred on February 6, 2006 or in April 2006; and
2. Entitlement to a non-economic loss (NEL) award for the residual impairment in the low back.

(iii) Background

- [4] The following are the basic facts.

- [5] The worker, born in 1977, was a miner working underground. He was injured on February 6, 2006, when he slipped on some ice in the employer's parking lot and wrenched his low back. Entitlement was granted for a low back musculoskeletal strain. He had a second injury in April 2006, in which his back seized up as he attempted to adjust the forks on a forklift. The April 2006 claim was allowed as a separate claim, on an aggravation basis.

- [6] The worker had episodes of low back pain in August 2015, and again on December 22, 2016. He claimed that these were recurrences of his 2006 low back injury, and sought entitlement for both incidents from the Board. He also sought a NEL award with respect to the permanent impairment (PI) in his low back.

- [7] In a decision letter dated January 20, 2017, entitlement was denied for both recurrences, and for a NEL for the low back, under both claims.

- [8] In the ARO decision dated November 2, 2017, the ARO found that there was no evidence to support that the worker's 2006 compensable accidents had caused or resulted in the degenerative changes that were evident in the 2006 x-ray and CT report. S/he reviewed the medical record, and concluded that there was no medical evidence of complaints of, or treatment for, low back pain between 2006 and an MRI of the low back in 2009, and nothing further in the medical record from 2009 to the claimed recurrence in August 2015. S/he noted that the diagnosis in 2016 was a disc herniation and protrusion at L4-L5, which was not compatible with the compensable diagnosis in 2006. Entitlement to the two claimed recurrences therefore was denied, as was entitlement to a NEL assessment.

- [9] The worker appeals from this decision.

(iv) Law and policy

[10] Since the worker claimed a recurrence of a prior compensable injury which occurred on February 6, 2006 and April 2006, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[11] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*.

[12] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages, Revision No. 9, would apply to the subject matter of this appeal:

- Package No. 60 – Recurrences;
- Package No. 212 – Determining MMR and PI; and
- Package No. 300 – Decision Making/Benefit of Doubt/Merits and Justice.

[13] Prior to November 1, 2014, Board policy on recurrences (*Operational Policy Manual* (OPM) Document No. 15-03-01) stipulates that workers are entitled to benefits for a recurrence of a work-related injury where there is clinical compatibility between the original injury and the current condition, or a combination of clinical compatibility and continuity. This policy goes on to provide further guidelines for evaluating the evidence of clinical compatibility and continuity:

Clinical compatibility

To establish clinical compatibility, a decision-maker compares the worker's current clinical condition to that following the initial accident. The decision-maker considers

- ☐ whether the parts of the body affected now are the same as, or related to, those affected initially
- ☐ whether the body functions affected now are the same as those affected initially, and
- ☐ the degree to which body functions are affected now (as compared to the effect of the initial condition).

Similar clinical conditions indicate that the current problem or problems may be a result of the original injury, whereas dissimilar or unrelated clinical conditions indicate that there is no compatibility, and therefore no recurrence.

Continuity

To establish continuity (i.e., a connection between the original clinical condition and the most recent problem or problems), the decision-maker considers whether the worker has

- ☐ complained to supervisors, co-workers, or health care practitioners on an ongoing basis since the original injury
- ☐ demonstrated ongoing symptoms since the original injury
- ☐ required work restrictions or job modifications
- ☐ had ongoing treatment for the original condition, or
- ☐ experienced a lifestyle change since the original accident (e.g., has the worker become unable to participate in household duties, or social or recreational activities?).

- [14] The final question in determining entitlement for a recurrence is whether there was a “significant new accident.” If there was a significant new accident, then the condition is not considered a recurrence of a prior workplace injury. The policy sets out the following “equations” as guidelines for adjudicators:

Insignificant new accident + compatibility + continuity = recurrence

An insignificant accident is one involving an ordinary or routine event, such as stooping to fasten a shoe or reaching for something on a shelf. If the accident is insignificant and clinical compatibility and continuity are established, the worker is entitled to benefits for a recurrence of the original injury.

Significant new work-related accident = new claim

A significant accident is one of some consequence or importance, such as a fall from a ladder. If there is a significant new work-related accident, a new claim is established.

A new claim is also established if the worker’s latest clinical problem or problems result entirely from a new accident.

(v) Analysis

- [15] In his submissions dated February 15, 2021 on behalf of the worker, Mr. Chartrand submitted that the testimonials he had provided from the worker’s supervisor and friends would satisfy the lack of continuity which the ARO had noted. He argued that it was more probable that the cause of the recurrences was the worker’s work history, rather than age-related degenerative changes at the time of the initial accidents in February and April 2006, when he was 29 years of age.
- [16] Mr. Hamilton provided submissions dated August 21, 2018, upon which he relies in this appeal. In those submissions, he stated that the employer agrees with the ARO decision, which should not be overturned.
- [17] As noted above, to establish a recurrence, there must be clinical compatibility, as well as continuity of complaint.

(a) Clinical compatibility

- [18] In terms of clinical compatibility, I note that, in the 2006 accident, the worker’s diagnosis was a musculoskeletal low back strain. In the Regional Evaluation Centre (REC) report dated June 20, 2006, the diagnosis was “mechanical back pain, facet strain and mild degenerative disc disease (DDD).” The DDD, of course, was not compensable.
- [19] I also note that there is a clinical note dated January 29, 2001 indicating that the worker had been having intermittent back pain, which the doctor diagnosed as mechanical back pain. There was no radiation of pain down the legs. The worker therefore had experienced at least one episode of low back pain prior to his compensable 2006 injuries.
- [20] In the August 2015 incident, the diagnosis was “acute over chronic lumbosacral dysfunction.” In the December 22, 2016 recurrence, the diagnosis was a disc herniation and protrusion at L4-L5, as a result of DDD.
- [21] Mr. Chartrand enlisted the assistance of Dr. Joel Anderson of the Occupational Health Clinics for Ontario Workers (OHCOW) Inc. Dr. Anderson was asked to review the worker’s file and to provide an opinion as to whether or not the worker’s back pain in 2015 and 2016 was

related to the 2006 workplace accidents. Mr. Trevor Schell, also of OHCOW, also authored a Retrospective Ergonomic Exposure Report, dated September 11, 2019, upon which Dr. Anderson relies heavily, and quotes from. Dr. Anderson's opinion was as follows:

This history from the start of his low back symptoms to present, imaging points to an initial wedging of L1 to L3 and disc lesion without impingement in 2006, with subsequent development (2016) of positive radiological findings of a large right paracentral disc protrusion and narrowing of the right lateral recess with the mass effect on L4. He developed an antalgic gait in the past 4 years as well.

The clinical and radiological progression of this disc lesion is confirmed by hospital records and MRI, as outlined above, in 2016 and 2017 ongoing.

A retrospective ergonomic assessment report was completed on September 11, 2019 by Mr. Trevor Schell and is attached.

This report details his work history [page 4-5], his workplace exposures [page 5-6] including physical demands of his job. The report concludes that:

In general, degeneration of the spine is more common among people engaged in heavy work than those in light work (Riihimäki, Tola, Videman, & Hanninen, 1988).

It can accelerate the degeneration up to 10 years (Shelerud, 2006). [The worker] is exposed to a number of risk factors contributing to back problems while working as a labourer and miner. These risk factors include but are not limited to acute loading, awkward postures, force demands, heavy physical work, and dynamic loading.

The report further goes on to discuss acute and chronic injuries sustained by [the worker] beginning in 2006 resulting in anterior wedging of L1 to L3.

The report states that once normal loading mechanisms have been damaged as is the case of [the worker], not only can the wedging continue to worsen but alteration in the spine makes it more susceptible to wear and tear even without being coupled with heavy lifting and awkward postures. [Page 8].

The report also outlines [the worker's] exposure to repetition and awkward posturing altering the spinal biomechanics with the development of anterior wedging that even when not in the presence of known occupational risk factors can worsen over time by itself.

This deterioration is demonstrated by radiological reports showing a worsening of [the worker's] spine until the point where he was no longer able to work.

This worsening is evidenced by the development of an antalgic gait which in itself also leads to altered spinal mechanics and disc degeneration, right paracentral disc extrusion, and degenerative changes pronounced between L3 and L4 with right sided disc protrusion.

Report [sic] further concludes that:

Given the radiology reports taken after the 2006 accident that showed multi level anterior wedging of the spine, subsequent altered spinal mechanics, and years of heavy lifting and prolonged awkward postures of the spine, it is safe to conclude that [the worker's] worsened spinal condition was indeed the result of the damage done from the 2006 accident and subsequent years of workplace exposures that altered the degeneration of his spine within a one year period due to the anterior wedging that may well

have taken decades if at all to reach this level of degeneration and impairment.

Conclusion

It is probable that workplace stressors, ongoing, from 2006, through the present, with worsening of radiological and clinical findings in 2016, are related to ongoing work stressors primarily and, to a lesser extent, identified non work related factors (see pp 9, Ergonomic report) which combined, probably resulted in the reoccurrence of his 2006 compensable low back condition.

[Emphasis added]

[22] Given the conclusions in Dr. Anderson's report, which relies heavily on Mr. Schell's opinion, it is worthwhile reviewing the medical diagnostics on file. They are as follows:

- April 14, 2006, Radiology - Mild anterior wedging of the L1 through L3 vertebral bodies. Findings are most marked at the L1 vertebral body. Anterior osteophytes at the L3/L4 level with associated mild disc space height loss at this level.
- June 7, 2006, CT scan - Axial, sagittal and coronal reformatted images of the lumbar spine have been obtained. There is minimal anterior and posterior osteophyte formation at the L3 L4 level. Schmorl formation is noted at the T12 L1, L1 L2 and L2 L3 levels. The bony canal is adequate at all lumbar levels with no compromise of the dural shadow. There is minimal annular bulging at the L4 L5 and L5 S1 levels.
- March 7, 2009, MRI - Small central disc bulge at L4-5 and L5-S1 level but without evidence of disc herniation, central canal stenosis or neural foramina compromise. Impression: Small disc bulges.
- October 30, 2015, MRI - Mild multilevel lumbar spondylosis as described, not associated with any significant spinal stenosis, no foraminal narrowing associated with spinal nerve root impingement. **Overall, the study has not significantly changed in comparison to the previous MRI from 2009.**
- June 19, 2016, MRI - Mild multilevel lumbar spondylosis as described, not associated with any significant spinal stenosis, no foraminal narrowing associated with spinal nerve root impingement. **Overall, the study has not significantly changed in comparison to the previously MRI from 2009.**
- December 27, 2016, MRI - Interpretation: **Multilevel lumbar spondylosis which has worsened in the interval, most notably at the L4-L5 level** where there is a large right paracentral disc extrusion with extruded disc material extending cranial to the intervertebral disc space by approximately 1.8cm and causing significant narrowing of the right lateral recess with mass effect on the traversing right-sided L4 nerve root.

[23] The April 14, 2006 x-ray report revealed anterior wedging from L1 to L3. However, a CT scan taken about two months later makes no mention of this; neither does the March 7, 2009 MRI. Further, while Dr. Anderson relies on Mr. Schell's opinion that studies have shown that workers engaged in heavier work develop degenerative changes in their spines earlier than those engaged in light work, and the acceleration can be up to 10 years faster, I note that the October 30, 2015 and June 19, 2016 MRIs indicate that, overall, there was no significant change in comparison to the MRI from 2009. One would think that, if the worker's lumbar spine degenerative changes were accelerated due to his heavy work, this would be reflected on the

investigations of the lumbar spine over time. However, this does not seem to be the case over this six-year period.

[24] I also note that Mr. Schell stated as follows regarding the findings in the 2006 scans:

At age 29, [the worker] experienced an acute injury to his lumbar spine in February 2006 when he slipped and fell on ice. In April of the same year, he experienced back pain when adjusting the forks on a forklift. Imaging from April 2006 shoulder found mild anterior wedging of the L1 through L3 vertebral bodies and minimal annular bulging from L4-S1. **This is commonly known as a wedge fracture.** Anterior wedging refers to the fact that the normally rectangular shaped vertebrae appears on radiological exams as triangular in shaped [sic] towards the front of the disc while the posterior section of the disc remains normal in appearance.

...

Wedge fractures are either caused by acute trauma such as falling and landing on the buttocks, repeated trauma due to wear and tear or due to conditions such as osteoporosis or Scheuermann's disease. [The worker] was never diagnosed with either condition. Leaving only the acute and repeated trauma.

[Emphasis added]

[25] I note that there is no indication in the Case Record that the worker was ever diagnosed with a wedge fracture. In addition, the worker was examined by Physiatrist Dr. Graham on June 20, 2006, who noted the wedging, but made no such diagnosis. Rather, his diagnosis was mechanical low back pain and facet strain. Further, in the February 2006 incident, in the Worker's Report of Injury (Form 6) dated February 23, 2006, the worker stated as follows regarding the mechanism of accident:

While walking outside I slipped on ice twisting my back.

[26] The employer, in the Form 7 dated February 15, 2006, added some further details:

Worker stepped off of forklift, slipped on some ice, twisting his low back.

[27] In the Multidisciplinary Health Assessment report dated June 20, 2006, Physiatrist Dr. Graham stated as follows:

... he was walking around on the surface at work and he slipped on the ice and caught himself, but in so doing, he twisted his back and developed a sudden stabbing pain in the right low back area. ...

[28] There is a clinical note dated February 22, 2006, which states as follows:

Slip and ~~fell~~ caught self at work on February 7.

- On ice, jarring back.

[29] It appears that the worker's doctor originally thought he had fallen, but this was corrected upon further questioning. This is in agreement with Dr. Graham's reporting and the incident descriptions in the Form 6 and the Form 7. I conclude that he did not fall in the February 2006 incident.

[30] In the April 2006 incident, the accepted accident history, as listed on the Employer's Report of Injury (Form 7) dated April 14, 2008, is that the worker was "adjusting the forks on the forklift when he felt pain to his lower right lumbar area." Again, there is no record of a fall.

[31] I therefore am not persuaded by Mr. Schell's theory that the worker had a wedge fracture, caused by a fall on the buttocks, or, given the lack of progression of the wedging between 2009 and 2016, by repeated trauma due to wear and tear.

[32] In summary, given the above misgivings regarding the two OHCOW reports, I do not find them persuasive.

[33] In this case, the two diagnoses are completely different. In 2006, the compensable diagnoses were mechanical back pain and facet strain, while in 2016, the diagnosis was an L4-L5 disc herniation. I therefore am unable to find that clinical compatibility is established for either episode of low back pain in 2015 or in 2016.

(b) Continuity of complaint

[34] In terms of continuity of complaint, I note that the worker advised the Emergency Room physician, Dr. Daniels, on December 23, 2016 that he had a 12-year history of chronic low back pain secondary to a fall at work. Neurosurgeon Dr. DeMarchi also noted this history in his report dated December 27, 2016, in which he recommended conservative treatment, rather than surgery, for the worker's low back and right leg pain.

[35] However, the medical record from the worker's family physician, Dr. Kauffeldt, does not corroborate these statements. There are notations of back pain on February 22, 2006, and April 8, 2006. In the latter, it is noted that the worker's back seized up at work, and he was taken to hospital by ambulance. This incident was allowed as a separate accident, on an aggravation basis. The worker was given three shifts off work, and returned to modified work. An entry dated April 11, 2006 indicates that his back pain was improving. The May 24, 2006 entry states that the worker continued on modified work for back pain; he was undergoing physiotherapy, and was awaiting a CT scan.

[36] In a report dated April 10, 2007, Chiropractor Dr. Chousky stated that he had examined the worker on April 9, 2007, at which time the worker had severe low back pain, had an antalgic posture, and walked with a tentative gait. Based on his examination, he felt that the worker's back and gluteal symptoms were contributed by disc injury and/or a lower lumbar sprained facet capsule with L4-L5 facet subluxation, for which he recommended Vax-d decompression therapy.

[37] An MRI of the lumbar spine dated March 7, 2009 revealed small disc bulges at L4-5 and L5-S1 levels but without evidence of disc herniation, central canal stenosis, or neural foramina compromise.

[38] There is nothing further on file with respect to medical reporting of low back pain until the Worker's Continuity Report dated August 24, 2015. In that report, the worker stated that the recurrence had occurred on August 9, 2015. He stated that he had been off work from the 14th to the 18th of August.

[39] There is also a Low Back Injuries Program of Care Initial Assessment Report dated August 20, 2015. In that form, Physiotherapist L. Robidaux stated that the worker had "acute over chronic lumbosacral dysfunction." Under "History of injury," she stated as follows:

Slip and fall at work February 14, 2006 and twisted back. Never fully resolved, lived with daily pain. Progressively worse over the years. August 10, 2015 woke up after night shift working on scoop & was more stiff than usual. This worsened over next 3 days.

[40] Thereafter, there is a clinical note dated June 21, 2016, when Dr. Kauffeldt reviewed the worker's 2006 compensation claim, in the context of his reports of current back pain. She noted as follows regarding his pain at that time:

Recently, returned to work w restriction, Jan 2016 assessment, deficits reviewed, currently modified duties.

Driving truck with lower back pressure.

Toronto assessment to clinic.

Pt feels fullness and pain with prolonged sitting.

Unable to get comfortable, sleep is restless.

Pt describes constant pain, sharp stabbing pain changes in intensity.

Pain localized to R lower side.

No LE weakness.

...

[41] An October 20, 2016 clinical note indicates that the worker was experiencing intermittent low back pain. He was having difficulty getting out of his truck, and could not throw a ball because of back pain.

[42] Thereafter, there are records relating to the worker's hospitalization following his visit to the Emergency Room on December 23, 2016. He was readmitted, for pain control related to his low back problems, on January 30, 2017.

[43] Thus, there is a gap in medical documentation of back pain between April 10, 2007 and the CT scan on March 7, 2009, and again from the CT scan to August 20, 2015, when the worker had the claimed 2015 recurrence.

[44] In an attempt to address that gap in the reporting, Mr. Hamilton obtained statements from a number of people, as follows:

- The worker's supervisor, KW, stated that he had filled out continuity forms for the worker on at least two occasions between 2007 and 2009.
- The worker's daughter stated as follows:

... [M]y dad hurt his back in 2006 my dad [name] has not been able to do anything with us his kids [name] and [name], so like sports, basketball, soccer, throw the football, swim and ect. [sic] Christmas of 2016 was the worst experience of my life. My dad was not home for Christmas he was in the hospital. Not just my family having to spend Christmas in the hospital with him. He couldn't spend it with the rest of his family. My brother and I had to leave our friends and school because we could not afford to keep our house in the valley because my dad could not go to work because of his back injury. ...

- Friends WW and RW wrote that the worker used to come to help them out in their home at least twice a year, as WW had had a stroke in 1996, and was unable to do some things around the house. Their statement indicates as follows:

We always knew [the worker] was available even though we were near [small town in Muskoka] and he was in [city in northern Ontario]. He would help fix equipment, do lawn work, anything that was needed.

Then [the worker] started to have issues with his health.

It has gotten so bad that he cannot even drive to our house let alone help when he gets here.

He use to be the "go to guy" but not anymore.

He is too proud to ask for return favours from all those he has helped in the past.

He has a heart of gold but his heart is being broken.

Depression has set in.

[45] Unfortunately, I do not find those notes to be persuasive with respect to filling the gaps in continuity of complaint. WW and RW do not indicate when the worker's health issues began, and whether they were related to the low back. They do not state when the worker stopped going to their house to assist them.

[46] Similarly, while the worker's daughter states that the worker injured his back in 2006, the majority of her statement appears to be focused on the period after 2016. Further, it appears that the children were in high school as of the 2016 injury, and thus, at the time of the 2006 injury, they would have been at a young age when playing basketball and throwing footballs, for example, would not have been something they would be involved in with their father.

[47] The worker's supervisor provides some evidence pointing to continuity from 2006 to 2009, but nothing thereafter.

[48] Given all of the above, I conclude that there is no continuity of complaint in this case.

[49] It follows that there is no entitlement for either of the claimed recurrences, and no entitlement to a NEL assessment.

DISPOSITION

[50] The appeal is denied.

DATED: August 12, 2021

SIGNED: S. Peckover